

Healthcare of Potential

Between Potential and Ideal - split chapter

Healthcare of Potential

Body, pain, diagnosis, and care that is not ownership of life.

The problem healthcare poses to the theory

Healthcare is where “possibility is not goodness” receives a body. The possibility to test, diagnose, operate, monitor, sedate, extend, or intervene is not enough. The question is how to distinguish a biologically possible action from care worthy of a living person.

The symptoms are queues, overload, protocols, tests, over-treatment, under-treatment, triage, end of life, AI diagnosis, and caregiver burnout. The root diagnosis: medicine fails when the possibility to intervene is confused with the worthiness of intervention, or when the medical metric replaces the person living in the measured body. Map of worthy care

Worthy care = medical knowledge × accurate diagnosis × listening × informed consent × proportionality × trust / system overload × alienation × blind protocol × fear × loss of subjecthood × error.

The map asks: is the goal cure, extension, relief, functioning, prevention, or accompaniment? Who bears the cost? What does the patient understand and want? What does the protocol see, and what does it miss?

End of life is a central test: not every possibility of extending biological function is an expansion of worthy life.

Triage is an optimum under scarcity, not an ideal. It must not become an excuse for leaving scarcity unrepaired.

AI can help diagnosis, but it is neither patient, nor physician, nor responsibility. What healthcare returns to the theory

Healthcare clarifies the difference between life as possibility and life as ideal. It also adds the notion of a clinical horizon: a window in which action can still alter the course of life. After it, the medical truth may still exist, but that intervention is no longer accessible in the same form.